

Radiotherapy for testicular cancer

Radiotherapy uses high-energy rays to destroy cancer cells. It is sometimes used to treat testicular seminomas and more rarely, non-seminomas.

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What is radiotherapy?

Radiotherapy is a treatment that uses high-energy rays to destroy cancer cells, while doing as little harm as possible to normal cells.

You may have radiotherapy for testicular cancer to treat a [seminoma](#) that has spread to the lymph nodes at the back of the tummy (abdomen). These are called the retroperitoneal lymph nodes. Radiotherapy aims to reduce the small risk of the cancer returning in this area of the body. You may have it after chemotherapy treatment if you have a stage 2 or 3 seminoma. We have more information on [staging of testicular cancer](#).

If you are not able to have chemotherapy, you may have radiotherapy on its own.

If a seminoma comes back, you may have radiotherapy with [chemotherapy](#).

More rarely, you may have radiotherapy to treat [non-seminomas](#).

Your doctor will talk to you about what the best treatment options might be in your situation.

Having radiotherapy for testicular cancer

You usually have radiotherapy treatment in the hospital radiotherapy department. You have it as a series of short daily treatment sessions. The sessions usually happen between Monday to Friday, with a rest at the weekend. Each session takes 10 to 15 minutes. Your doctor will discuss the treatment and possible side effects with you.

A course of radiotherapy for seminoma may last about 2 to 3 weeks. You usually have treatment as an outpatient. The person who operates the machine is called a radiographer. They give you information and support during your treatment.

Radiotherapy does not make you radioactive. It is safe for you to be with other people, including children, throughout your treatment.

Temporary effects on fertility

There is no evidence that radiotherapy has any effect on children you may have (conceive) after treatment. But because radiotherapy may have a temporary effect on the sperm, you should use contraception during treatment and for 6 to 12 months after treatment. This allows time for your sperm to recover from any damage treatment may have caused.

Contraception

During radiotherapy, a small dose of radiation reaches the remaining testicle. This may temporarily affect your sperm. So you will be advised to use effective contraception during treatment.

There is no evidence that radiotherapy has any effect on children you have after treatment. But you are usually advised to use contraception for 6 to 12 months after treatment. This allows time for your sperm to recover from any damage that treatment may have caused.

Treatment sessions

At the beginning of each session of radiotherapy, the radiographer will position you carefully and make sure you are comfortable. During your treatment, you will be alone in the room. But you can talk to the radiographer, who will watch you from the next room.

Radiotherapy is not painful, but you will have to lie still for a few minutes during the treatment.

Planning your radiotherapy

You will have a hospital appointment to plan your treatment. You will usually have a [CT scan](#) of the area to be treated. During the scan, you need to lie in the position that you will be in for your radiotherapy treatment.

Your radiotherapy team use information from this scan to plan:

- the dose of radiotherapy
- the area to be treated.

You may have some small, permanent markings made on your skin. The marks are about the size of a pinpoint. They help the radiographer make sure you are in the correct position for each session of radiotherapy.

These marks will only be made with your permission. If you are worried about them, talk to your radiographer.

Side effects

Radiotherapy to your tummy can cause side effects. But these can usually be controlled with medicines. Your doctor or specialist nurse will tell you more about what to expect. These side effects usually disappear gradually once your course of treatment has finished.

It is important to let your doctor know if you are having any problems with side effects. Most side effects are mild, and you can have medicines to treat them successfully.

Skin changes

If you have fair skin, the treated area may become red. If you have black or brown skin, it may become darker. But after your treatment finishes, these changes should go away. Your doctor can advise you how to look after your skin and prescribe cream if your skin is sore.

Feeling sick (nausea)

Radiotherapy to the tummy area may make you feel sick. Your doctor can prescribe anti-sickness medicine to prevent or stop this. Let your doctor know if the tablets are not working for you. There are other medicines they can prescribe.

Tiredness

Radiotherapy often makes people [feel tired](#), especially towards the end of treatment. Try to pace yourself and avoid doing anything that you do not really need to do. Gentle exercise, such as short walks, can help to improve tiredness. It is good to balance this with plenty of rest.

Sometimes, tiredness can last about 8 weeks or longer after treatment finishes. Your energy levels should then slowly improve.

Diarrhoea

You might get diarrhoea, but your doctor can prescribe you medicines to control this. Let your doctor know if you have diarrhoea. It is important to drink plenty of fluids. Try to eat fewer high-fibre foods, such as fruit, vegetables, beans, pulses and wholewheat cereals.

Possible late effects of treatment for testicular cancer

Effects on the heart

Radiotherapy to the lymph nodes at the back of the tummy may have an effect on your heart many years after treatment has finished. This is called a late effect. This is because these lymph nodes are positioned near to the heart. You may have regular follow-up appointments to check your heart health

after cancer treatment. The benefits of having treatment will usually far outweigh this risk. Your specialist doctor can tell you more about this.

Second cancer

Radiotherapy may slightly increase the risk of developing another cancer later. This does not mean that you will definitely develop another cancer. The benefits of having treatment will usually far outweigh this risk. Talk to your specialist doctor if you have any concerns about this.

About our information

References



Below is a sample of the sources used in our testicular cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

The British Association of Urological Surgeons (BAUS) Guidelines on Testicular Cancer. March 2015. Available from: https://www.baus.org.uk/professionals/sections/testicular_cancer.aspx (accessed April 2022).

European Association of Urology (EAU) Guidelines on Testicular Cancer 2022. Available from: <https://d56bochluxqnz.cloudfront.net/documents/full-guideline/EAU-Guidelines-on-Testicular-Cancer-2022.pdf> (accessed April 2022).

European Society for Medical Oncology (ESMO). Testicular seminoma and non-seminoma: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. Last updated 2022.

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Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Dr Ursula McGovern](#), Consultant Medical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

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⚠ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help